

Cognitive Behaviour Therapy (CBT) for menopausal symptoms

The guideline *Menopause: Identification and Management* (National Institute for Health and Care Excellence, NICE 2015, updated 2024)¹ recommends that GPs and healthcare professionals give information and advice to menopausal women about cognitive behaviour therapy (CBT).

Anxiety and stress

NICE recommends CBT as a treatment option for anxiety experienced by women during the menopause transition and post menopause.¹ Women can feel anxious at this time, due to unpredictable hot flushes and social embarrassment, which can lead to avoidance of social and other activities. Hot flushes can be associated with palpitations. Bearing these issues in mind, CBT can be adapted from protocols for social and general anxiety.



Depressed mood

The NICE update on menopause (2024)¹ recommends menopause-specific CBT as a treatment option for women experiencing depressed mood, during the menopause transition and post menopause, that does not meet the threshold for clinical depression, particularly when these symptoms occur alongside hot flushes and night sweats or vasomotor symptoms (VMS). In such cases, CBT can be offered alongside other treatments, including hormone replacement therapy (HRT), or as an alternative for those who are unable or choose not to use other treatment options.

For women who are suspected of having, or have been diagnosed with, depression, the 2024 NICE menopause guidelines should be used in conjunction with the NICE guideline on depression in adults, to ensure comprehensive care.

Past depression is the main predictor of depressed mood during the menopause transition. About 10% of women are more likely to have depressed mood during the menopause transition, which tends to be time-limited, and there are two-way interactions between mood and hot flushes and night sweats. Going through the menopause transition can affect self-esteem and lead to low mood as a result of negative beliefs about menopause and stigma about age and reproductive status, as well as due to hot flushes affecting sleep and psychosocial stresses which often accumulate during mid-life. Some women report premenstrual type symptoms which may be due to fluctuating hormone changes. Typically, there are interactions between the above factors when a woman seeks help. Taking these factors into account, CBT for depressed mood can be offered.

Hot flushes and night sweats

CBT developed specifically for menopausal symptoms can help women to manage vasomotor symptoms and has been found to be effective in clinical trials for women going through the menopause and for breast cancer patients². Improvements were maintained at six-month follow-up and there were additional benefits to quality of life. The CBT approach is theory based and focuses on stress and wellbeing, hot flushes, night sweats and sleep problems, over 4-6 weeks; it is available in a self-help book³ and small group formats⁴. The North American Menopause Society (2023)⁵ recommends CBT as an effective non-hormonal treatment option for hot flushes and night sweats. NICE (2024)¹ advises healthcare professionals to consider menopause-specific CBT for managing VMS – either alongside HRT, or as an option for individuals who cannot or prefer not to use HRT.

Sleep problems and insomnia

NICE (2024)¹ also recommends that healthcare professionals consider menopause-specific CBT for women experiencing sleep disturbances that commonly occur during the menopause transition. Night sweats tend to wake women and often disrupt sleep. Developing good sleep habits and an automatic routine if woken up by night sweats can help, as can the CBT-i approach that has been found to be effective for insomnia⁶.

Who can provide CBT?

Primary care healthcare professionals (e.g. GPs, counsellors, psychological well-being practitioners and trained nurses) can provide these low intensity therapies. Women can also use self-help books, with some guidance, and return for an appointment after 4-6 weeks to review progress.

References

1. National Institute for Health and Care Excellence, Menopause: identification and management guideline [NG23], update 2024. <https://www.nice.org.uk/guidance/ng23>
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3. Hunter MS & Smith M. Managing hot flushes and night sweats: a cognitive behavioural approach to menopause, Routledge 2014.
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5. Nonhormonal therapy Position Statement of The North American Menopause Society. *Menopause* 2023. 30 (6,) 573-590.
6. Espie CA. "Stepped Care": A health technology solution for delivering cognitive behavioral therapy as a First Line Insomnia Treatment. *SLEEP* 2009;32(12):1549-1558.

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This BMS Tool for Clinicians will be updated when guidance changes and/or new data becomes available.

British Menopause Society

Educates, informs and guides healthcare professionals, working in both primary and secondary care, on menopause and all aspects of post reproductive health.

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